

Policy on Professional Functioning of the Organisation

«Hospital Name»

Document control

Document No.	HCO/ROM/POL/05	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Medical Superintendent Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» functions professionally. It maintains written strategic and operational plans aligned to its vision, mission and values, developed in consultation with stakeholders. It coordinates its functioning with departments and external agencies, monitors progress toward defined goals, prepares and reviews an annual budget, reviews its committees for effectiveness, tracks measurable service standards, and has written guidance for managing change.

1. Purpose

This policy explains how «Hospital Name» develops and maintains strategic and operational plans aligned to its vision, mission and values in consultation with stakeholders; coordinates and monitors progress toward those plans with departments and external agencies; prepares and reviews an annual budget; reviews the effectiveness of its committees; documents and monitors measurable service standards; and maintains written guidance for change management.

This policy does not cover patient assessment, clinical care, medication management, patient rights, infection control, or quality and safety monitoring — those are covered in other hospital policies.

2. Scope

This policy applies to those responsible for governance, the Medical Superintendent, the Quality Coordinator, departmental leaders, committee chairs, and staff whose roles involve strategic planning, budgeting, or committee participation at «Hospital Name».

3. Policy standards

«Hospital Name» maintains documented strategic and operational plans with measurable goals; coordinates their implementation with departments and external agencies; prepares an annual budget that covers those plans and includes an adequate allocation for IPC and quality improvement activities; reviews the effectiveness of each committee it maintains; documents and monitors measurable service standards; and keeps written guidance for managing change.

Staff follow the written guidance below, keep the records it requires, and raise concerns through the channels it describes.

4. Non-negotiable rules

1. The hospital must have written strategic and operational plans — a verbal commitment or a presentation slide is not a plan.
2. Goals and objectives drawn from those plans must have measurable outcomes and defined time frames — goals without metrics cannot be monitored.
3. The annual budget must include an adequate allocation for IPC activities and quality improvement activities — a budget that makes no provision for these is incomplete.
4. The leadership must formally review the effectiveness of each committee the hospital maintains and document the outcome — a committee that has never been reviewed does not comply with this standard.
5. Minutes must be maintained for each committee meeting — a meeting with no minutes has no record of decisions or actions.
6. Service standards must be documented as measurable benchmarks and monitored at a defined frequency — standards that are neither measurable nor actively monitored are not compliant.
7. Staff who see any of these rules broken report it the same shift to the department in-charge or the Medical Superintendent.

5. What we do

5.1 Develop and maintain strategic and operational plans

The leader of «Hospital Name» defines and develops the process for producing strategic and operational plans. The strategic plan is aligned to the organisation's vision and mission statement and is consistent with its values. It is developed in consultation with all stakeholders, including the community the organisation serves.

The strategic plan takes into account both an external and internal scan — for example using SWOT analysis (Strengths, Weaknesses, Opportunities, Threats). Inputs considered in finalising the plan include the findings from the hospital's risk management programme, patient safety goals, and the results of facility rounds. Both the strategic plan and each operational plan have defined goals and objectives that are measurable and carry defined time frames.

The operational plan is produced on an annual basis. Both plans are documented and retained on file.

5.2 Coordinate functioning and monitor progress

Goals and objectives are drawn from the strategic and operational plans. They are consistent with the hospital's mission and values and have measurable outcomes.

«Hospital Name» coordinates its functioning with its departments and with external agencies as applicable. Progress toward the defined goals and objectives is monitored at regular intervals. Where a goal has not been achieved, the reasons are analysed and appropriate action is taken. This may be done through periodic reviews and/or formal management review meetings, which are documented.

A record is kept of the analysis and the action taken for each goal that was not achieved.

5.3 Plan and budget annually

«Hospital Name» plans and budgets for its activities each year. Budget development is guided by the hospital's strategic and operational plans. The budget development and review process includes a comparison of budgeted expenditure against actual expenditure. Responsibilities for budget development and review are clearly defined and followed.

An adequate budget is allocated for infection prevention and control activities and for quality improvement activities. What constitutes an adequate allocation is for the hospital to determine in light of its own IPC programme and quality improvement activities — the source requirement is that the allocation is adequate, not that it meets a prescribed percentage or amount. The hospital may organise its budget on a calendar year (January—December) or a financial year (April—March) basis.

The budget is documented and approved. The form it takes is for the hospital to define — the obligation is that it covers the hospital's activities and includes provision for IPC and quality improvement.

5.4 Review committee effectiveness

The leadership of «Hospital Name» reviews the functioning of each committee for effectiveness. The review considers: whether the purpose for which the committee was established is being met; whether the committee is meeting at its prescribed frequency; whether it is identifying issues and suggesting remedial measures; and whether corrective

and preventive actions it has recommended are being adequately monitored by way of risk mitigation within the committee's scope.

Minutes are maintained for each committee meeting. The Guidebook notes that, to support an effective review, the organisation may document the scope of every committee, the roles and responsibilities assigned to its members, and the prescribed meeting frequency — the organisation decides whether and how to do this.

5.5 Document and monitor service standards

«Hospital Name» develops measurable benchmarks for the different aspects of the services it provides. The benchmarks may be derived from the organisation's values, from national guidelines, or from the hospital's own experience. The hospital defines the person responsible for monitoring each benchmark and the frequency at which monitoring is done.

Service standards are documented. Monitoring results are recorded.

5.6 Written guidance for change management

«Hospital Name» maintains written guidance for managing change within the organisation. This guidance covers operational, financial, and departmental changes, and addresses succession planning and changes in leadership.

The written guidance addresses: how the change will be communicated across the organisation; who owns the change; and how the cultural aspects of the transition will be managed.

This is an Excellence-level objective element, assessed during re-accreditation. The hospital keeps a record of significant changes managed under this guidance.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and properly resourced.
Medical Superintendent	Owns the strategic plan, operational plans, annual budget, committee records and day-to-day records for this policy.
Those responsible for governance	Approve the strategic plan and annual budget; receive and act on committee effectiveness reviews.
Quality Coordinator	Audits this policy periodically; keeps training records; coordinates management review meetings; supports governance packs this policy requires.
Departmental leaders	Participate in strategic and operational planning; manage their department's contribution to defined goals; participate in committees as assigned.

7. Quality monitoring

The Quality Coordinator audits this policy periodically, checking a sample of records against each of the What-we-do steps.

Documentary evidence is on file for each asterisked objective element (marked * in the Level column of Section 11).

If an audit finds a gap that is still open after a defined timeframe, the hospital carries out a root-cause analysis and records the findings and corrective actions.

This policy is reviewed every year, and sooner if the strategic plan, committee structure, or budget process changes significantly.

8. Training and staff acknowledgement

Staff covered by this policy are trained when they join, and again every year after that. Training covers the What-we-do steps and the non-negotiable rules.

I have read the Policy on Professional Functioning of the Organisation of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Medical Superintendent; those responsible for governance; Quality Coordinator; departmental leaders; committee chairs; staff covered by this policy

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
IPC	Infection Prevention and Control
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
ROM	Responsibilities of Management (NABH Hospitals 6th Edition chapter)
SWOT	Strengths, Weaknesses, Opportunities,

	Threats
--	---------

11. Traceability to NABH HCO Full Accreditation 6th Edition ROM.5

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
ROM.5.a	Commitment	The organisation has strategic and operational plans, including long-term and short-term goals commensurate to the organisation's vision, mission and values in consultation with the various stakeholders.	Section 3; 5.1	Medical Superintendent
ROM.5.b	Commitment*	The organisation coordinates the functioning with departments and external agencies and monitors the progress in achieving the defined goals and objectives.	Section 3; 5.2	Medical Superintendent
ROM.5.c	Commitment	The organisation plans and budgets for its activities annually.	Section 3; 5.3	Medical Superintendent
ROM.5.d	Achievement	The functioning of committees is reviewed for their	Section 3; 5.4	Medical Superintendent

		effectiveness.		
ROM.5.e	Commitment*	The organisation documents the service standards that are measurable and monitors them.	Section 3; 5.5	Medical Superintendent
ROM.5.f	Excellence*	Systems and processes are in place for change management.	Section 3; 5.6	Medical Superintendent

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

ROM.5.a — Strategic and operational plans

- Written strategic plan (vision/mission aligned, stakeholder consultation record, measurable goals with time frames).
- Annual operational plan(s) drawn from the strategic plan.

ROM.5.b — Goal coordination and monitoring

- Progress-review records against strategic and operational plan goals (periodic reviews and/or management review meeting minutes).
- Analysis record for each goal not achieved, with the action taken.

ROM.5.c — Annual budget

- Annual budget document covering the hospital's activities.
- Budget-vs-actual review record.
- Evidence of an adequate allocation for IPC activities and quality improvement within the budget.

ROM.5.d — Committee effectiveness

- Minutes for each committee meeting.
- Leadership review record assessing each committee's effectiveness.

ROM.5.e — Service standards

- Documented service-standard benchmarks (measurable, with responsible person and monitoring frequency defined).
- Monitoring records for each benchmark.

ROM.5.f — Change management

- Written change-management guidance document.
- Record of significant changes managed under the guidance.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Responsibilities of Management, standard ROM.5.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — ROM.5 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.